

Swallowed Foreign Body - Female (Adult)

Algorithm 1274 | Adult (18 years and older) | Gender at birth: Female

Definition

- UAT-only emergency and in-person assessment pathway for suspected ingestion of a non-food object in an adult, child, or infant; not approved for real-patient or production use.
- Adult patient.
- Female.

Initial assessment

1. [YES_NO] Is the patient choking nowâ€”unable to speak or cry, unable to cough effectively, struggling or unable to breathe, becoming blue or grey, or becoming unresponsive?
2. [OPEN_TEXT] What is the patient's exact age, what object may have been swallowed, how many, and what are its approximate size, shape, and material?
3. [DURATION] When was the object swallowed or last seen, and was the ingestion witnessed or only suspected because an item is missing?
4. [YES_NO] Could it be a button or coin battery, one or more magnets, a sharp or pointed object, a large or long object, a lead-containing or toxic object, an expanding water bead, or an unknown object?
5. [YES_NO] Is there drooling, difficulty or pain swallowing, refusal to feed, neck or chest pain, coughing, gagging, vomiting, abdominal pain or swelling, blood in vomit or stool, fever, or unusual drowsiness?
6. [YES_NO] If the patient is under 18, can they speak privately now, is the accompanying caregiver considered safe, and is there an approved safe callback method if contact is lost?

Triage and redirect questions

1. Redirect → Poisoning

Was a medicine, household chemical, plant, or other potentially poisonous substance swallowed rather than a solid object?

A potentially toxic ingestion belongs in the Poisoning pathway.

1. Disposition level 100

If the patient is under 18, is harm occurring now, is there immediate danger from an accompanying or suspected caregiver, could the child be returned now to an unsafe person or place, or did contact end while an immediate threat remained unresolved?

Immediate danger, ongoing harm, an imminent unsafe return, or loss of contact during an unresolved immediate threat requires emergency action. Qatar 999 is the verified emergency route. Exact child-protection reporting, consent, documentation, and information-sharing requirements are GOVERNANCE_REQUIRED.

2. Disposition level 78

If the patient is under 18 and there is no confirmed immediate danger, are they unable to speak privately, is caregiver safety uncertain, or is there another safeguarding concern requiring trained assessment?

An inability to obtain a private history, uncertainty about caregiver safety, or another safeguarding concern needs prompt trained assessment without automatically treating it as a confirmed emergency. The exact Qatar safeguarding pathway is GOVERNANCE_REQUIRED.

3. Disposition level 100

Is there severe choking: an ineffective or absent cough, inability to speak or cry, severe breathing difficulty, blue or grey colour, or reduced responsiveness?

Foreign-body airway obstruction is immediately life-threatening. HMC identifies choking as a reason to call Qatar 999, and 2025 Resuscitation Council UK guidance specifies age-dependent choking first aid.

3. Disposition level 100

Even without symptoms, is there suspected or confirmed ingestion of a button or coin battery; any magnet, especially more than one or a magnet with metal; a sharp or pointed object; a large or long object; a lead-containing, toxic, caustic, or unknown object; an expanding water bead; or is there drooling, difficulty swallowing, refusal to feed, neck/chest/abdominal pain, persistent cough, vomiting, bleeding, fever, or unusual drowsiness?

Button batteries can cause rapid tissue injury without early symptoms. Magnets can attract across bowel walls, and sharp, large, long, toxic, expanding, or symptomatic objects require urgent location and complication assessment. RCPCH/RCEM advises radiographs when battery, magnet, or sharp-object ingestion is possible.

3. Disposition level 70

After the emergency branches are excluded, was any other small blunt object swallowed or possibly swallowed, with the patient currently breathing and swallowing normally and without symptoms?

Observation may be appropriate for selected small blunt objects, but only after a clinician establishes the object's identity, location, size relative to patient age, timing, symptoms, and follow-up reliability. This UAT pathway does not authorize telephone-only self-care.

Advice

Immediate child safeguarding danger

- Call Qatar 999 now when immediate danger or ongoing harm is suspected. If contact continues, confirm the child's current location and a safe way to communicate without alerting or confronting a suspected unsafe caregiver. If contact is lost, use only the approved safe callback method and follow the governance-approved emergency escalation process. Do not promise confidentiality. Exact Qatar child-protection reporting, consent, documentation, and information-sharing rules are GOVERNANCE_REQUIRED. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

- Call Back If: harm occurring now or immediate danger; imminent return to an unsafe person or place; contact lost while an immediate threat remains unresolved.

Urgent private child safeguarding assessment

- Keep questions neutral and seek a private conversation only when it is safe. Do not disclose sensitive information to a caregiver whose safety is uncertain, do not confront or alert a suspected unsafe person, and do not promise confidentiality. Arrange prompt in-person assessment through the governance-approved Qatar child safeguarding pathway and record only an approved safe callback method. If immediate danger emerges or contact is lost during an unresolved immediate threat, use the emergency branch and call Qatar 999. Exact reporting, consent, documentation, and information-sharing rules are GOVERNANCE_REQUIRED.
- Call Back If: private history cannot be obtained safely; caregiver safety remains uncertain; new immediate danger or loss of contact.

Severe choking “ call Qatar 999

- Call Qatar 999 immediately on speaker and follow the operator's instructions. If the patient can cough effectively, encourage coughing and monitor continuously. If the cough is ineffective, a trained or dispatcher-guided responder may give up to 5 back blows. If these fail, use up to 5 abdominal thrusts for an adult or child over 1 year, but use chest thrusts instead for an infant under 1 year or a pregnant patient; continue the age-appropriate sequence as directed. Do not perform a blind finger sweep or use an unapproved suction device. If the patient becomes unresponsive, start age-appropriate CPR immediately as directed by 999. Even after the object clears, a patient who received thrusts or chest compressions needs clinical assessment. Do not drive the patient to hospital. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: cough becomes ineffective; breathing remains difficult; reduced responsiveness.

High-risk swallowed object “ emergency assessment

- Call Qatar 999 for breathing difficulty, collapse, severe symptoms, or when emergency transport is advised; otherwise proceed immediately to the emergency department using the Qatar destination approved for this UAT environment. Do not wait for symptoms. Give no food, drink, honey, medicine, or laxative and do not induce vomiting unless Qatar 999 or a poison specialist gives object-specific instructions. Keep the patient observed. Bring the matching battery, magnet, object, packaging, container, or a photograph when safe, but do not delay departure. The patient may need urgent radiographs and specialist removal. Do not use MRI until retained metal or a magnet has been excluded by clinicians. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: drooling or difficulty swallowing; coughing, breathing difficulty, pain, vomiting, bleeding, fever, or drowsiness.

Other swallowed object “ in-person assessment

- Arrange prompt same-day in-person assessment through the Qatar destination approved for this UAT environment. Until a clinician confirms that observation is safe, do not induce vomiting, give laxatives, or give food, drink, or medicine. Do not search the stool or assume passage unless the treating clinician provides an object-specific monitoring plan. Escalate to Qatar 999 if choking, breathing difficulty, reduced responsiveness, severe pain, or collapse develops.
- Call Back If: difficulty swallowing or drooling; pain, vomiting, abdominal swelling, bleeding, fever, cough, or breathing symptoms.

First aid

- Call Qatar 999 now when immediate danger or ongoing harm is suspected. If contact continues, confirm the child's current location and a safe way to communicate without alerting or confronting a suspected unsafe caregiver. If contact is lost, use only the approved safe callback method and follow the governance-approved emergency escalation process. Do not promise confidentiality. Exact Qatar child-protection reporting, consent, documentation, and information-sharing rules are GOVERNANCE_REQUIRED. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
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References

- Hamad Medical Corporation, "Home Safety â€” Choking and Small Objects", <https://www.hamad.qa/EN/your%20health/Patient%20and%20Family%20Education%20Unit/Publications/Diseases-and-Care/Pediatrics/Pediatrics/Home%20Safety%20-%20Choking%20and%20Small%20Objects%20-%20English.pdf>
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- Resuscitation Council UK, "First Aid Guidelines â€” Choking in an adult", 2025, <https://www.resus.org.uk/professional-library/2025-resuscitation-guidelines/first-aid-guidelines>
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- UK Office for Product Safety and Standards, "Button batteries campaign: how to stay safe", <https://www.gov.uk/government/news/button-batteries-campaign-how-to-stay-safe>
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Provenance

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